

Mp10 Web — User Guide

Finding patients, working encounters, and everything the browser can do

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1 Mp10 Web — User Guide

This guide is for clinic staff using Mp10 Web in a browser. It does not cover how the system is installed or built — that is the **Mp10 Web — Installation** guide.

Two companion guides cover things this one only points at: **Mp10 Web — Printing** for the print helper, and **Mp10 Signature Pads** for the Topaz pad.

1.1 Signing in

Go to the Mp10 Web address your office gives you. Enter your username and password and select **Sign In**.

What you can see and do depends on your account. If a screen or button described below is missing, your account doesn't have permission for it — ask your administrator.

Changing your own password. At the bottom of the menu, beside the sign-out button, is a **key** (). It asks for your current password and the new one twice. The new password must be at least 8 characters.

Changing it **signs you out of Mp10 Web everywhere**, including the window you are in — that is deliberate, so a password you have just replaced cannot still be in use somewhere you have forgotten about. You are taken back to the sign-in page to sign in with the new one.

Your Mp10 Web password is separate from your Mp10 desktop password. Changing it here does not change the one you use in the desktop applications.

1.2 Getting around

After signing in you'll see a menu down the left. It only lists the areas you have permission to use, so two people may see different menus on the same system. The main areas are **Dashboard**, **Patients**, **Encounters** and **Modality worklist**.

The menu can be collapsed to icons with the control at its top, to give the grids more room. Mp10 Web remembers whether you left it collapsed.

1.2.1 What the grids remember

The three big screens — Patients, Encounters and Modality worklist — all behave the same way, and both kinds of memory below are per person, so two people sharing a computer keep their own.

Column widths and order are remembered for good. Drag a column edge to resize it, or drag a heading to move it, and it stays that way the next time you sign in — on that computer. If you get into a mess, **Reset layout** in the button bar puts the columns back to how they ship. It only affects columns; it does not clear your search or filters.

Filters are remembered until you close your browser. Search a name on Patients, go to Encounters, come back — your search and filters are still there, and the grid is showing what it was showing. Close the browser and they are gone; this is meant to save you retyping during a shift, not to follow you into next week.

The **Modality worklist's** date range follows a rule of its own, because that box is filled in for you rather than starting empty. **A date range you set yourself is remembered** like any other filter. One you never touched is not — it is reset to today each time, so a browser left open overnight opens on today's work rather than yesterday's. **Clear** puts it back to following the calendar.

1.3 The Dashboard

After signing in, the Dashboard is what you land on first. It has three cards:

- **Today** — a quick count of today’s encounters, split out by site. If today shows nothing yet — first thing in the morning, or on a weekend or holiday when the office is closed — that’s normal, not a fault: the card names the day it’s showing and also tells you the last day that actually had encounters, so you can tell “quiet today” from “something’s wrong” at a glance.
- **Needs attention** — encounters from the last several days that still need a signature. Use it to catch anything that was missed before it gets old.
- **System health** — three quick status rows:
 - **Print helper (this workstation)** — whether the print helper program is answering on **the computer you’re sitting at right now**, not on the server. It only knows about itself, so if you check this at the front desk and a colleague checks it at their own desk, you can each see a different answer — that’s expected, not a bug. If it shows a job stuck (“stopped responding”), see **When printing does not work**.
 - **HL7 interface** — when the last message came in from the lab/imaging interface, and how many are still waiting to be processed. This only turns red when messages have gone quiet for longer than expected — a large “unprocessed” count on its own is normal for a busy practice and does not turn the row red by itself.
 - **Modality worklist** — how many imaging orders are queued. Seeing “nothing scheduled” here is a normal, everyday answer, not an error — it just means nothing is queued right now.

1.4 Finding a patient

The patient list shows every patient in a scrollable grid, with search, filtering, sorting, and paging so you can find one quickly among the full list.

- **Search** — type a last name, first name, Social Security number, or record number, and press Enter. Partial names work.
- **Filter** — narrow the list by sex using the box above the grid, and by city or date of birth using the small funnel in those column headings. The City funnel takes more than one city at once, and has its own **Find...** box because the city list is long.
- **Sort** — select a column heading to sort by it; select again to reverse the order. Not every heading sorts — **Sex**, **Phone**, **Mobile** and **St** have no arrow, because sorting a list this size on those columns would be slow enough to be worse than useless.
- **Paging** — use the controls at the bottom to move through pages of results. The count beside them is the number of patients your search matched, not the number on screen.

Single-click selects a row; double-click opens it. Selecting is what turns on the buttons in the bar at the bottom — most of them act on the row you have chosen. The patient then opens **in a window on top of the list**, so closing it puts you back exactly where you were: same search, same page, same row highlighted.

On patients who share a surname. The list is ordered by last name, and within one surname the order is not alphabetical by first name — with over two thousand patients called RODRIGUEZ, sorting them all every time you opened the screen cost more than it was worth. Use the search box, which looks at first names too, rather than paging through a common surname.

1.4.1 The Created column

The last column, **Created**, is the date the patient record was first made. It sorts, so it is the quickest way to see who was registered recently.

1.5 The patient record

A patient’s record has four tabs across the top:

- **Demographics** — name, address, contact details, and the HIPAA signature.
- **Additional** — the further identifying details that do not fit on Demographics.
- **Insurance** — the patient’s insurance plans.

- **Encounters** — the patient’s visits.

Select a tab to switch to it.

The record normally opens **as a window on top of the patients list**, and **Close** returns you to the list untouched. It can also be opened as a full page of its own — a bookmark or a link someone sent you does that — and in that case there is a **Back to list** button instead, and the tab you are on is part of the address, so refreshing or sharing the link reopens on the same tab.

1.5.1 Demographics

This tab shows the patient’s personal information: name, address, phone, date of birth, and similar details.

- Select **Edit** to make changes, then **Save**.
- **Name and identity**: last name and first name are on one line, with date of birth (“DOB”) and Social Security number on the next, and sex and marital status below that. The patient ID card photo sits beside these fields (see “Capturing images” below).
- **Language is no longer editable** on this screen. It is not shown or changed here.
- **Zip code**: if you enter a zip code, the city and state fill in automatically — but only if those boxes were empty. If you already typed a city or state, autofill will not overwrite what you entered.
- **ID card photo**: you can attach a photo of the patient’s ID (see “Capturing images” below).
- **Mailing address** sits beside the **HIPAA signature**, with the two address lines stacked and the zip, city and state on the row underneath.

1.5.2 The HIPAA signature

The card to the right of the address shows the signature certifying that this **person** received the HIPAA notices. It is not the same thing as the per-visit signature on an encounter — that one is a receipt for a particular visit, this one is about the patient.

Underneath the signature it tells you **when it was given**. That is the point of the card: a signature on file is not much use if nobody can say when it was signed.

Three things you may see:

- a signature and a date — the normal case;
- **“No HIPAA signature on file for this patient”** — nothing has been captured;
- a signature and **“Signed on an unrecorded date”** — the signature is real, but it predates the system recording signing times. It is not an error and there is nothing to fix.

Capture signature (or **Re-capture signature**) takes a new one from a signature pad. It is pad-only by design — a consent cannot be uploaded from a file, it has to be signed. See **Patient signatures from a signature pad**.

1.5.3 Insurance

This tab shows a list of the patient’s insurance plans, plus a form below the list with details for whichever plan you select.

- Select **New** to add a plan, fill in the form, and select **Save**.
- Select an existing plan in the list to view or edit it.
- Select **Delete** to remove a plan.
- Select **Cancel** to discard unsaved changes in the form.
- Each plan can have a photo of the insurance card attached (see “Capturing images” below).

Primary, Secondary, and Historical — every plan is marked as one of these three. Primary and Secondary are the patient’s active coverage; Historical means the plan is on file but no longer active. There is no separate “active” checkbox — this Primary/Secondary/Historical setting is what determines whether a plan counts as active.

If you see “**This record changed since you loaded it. Reload and try again.**” — this means someone else (on the web or on a desktop screen) saved a change to this same insurance record while you had it open. This is expected behavior, not an error in the system. **Do not simply try to save again.** Reload the patient’s insurance tab first, so you’re looking at the current version, then make your change again. Saving blindly on top of someone else’s change can create a duplicate record instead of updating the one you meant to.

1.5.4 Encounters

This tab lists the patient’s visits (encounters), newest first.

- Filter by date range, insurance, or whether the encounter is still open.
- Select **New** to add an encounter, or select an existing one to edit it. Either one opens in a dialog on top of the patient record, so you never leave the patient’s page just to work on an encounter.
- The action bar at the bottom of the grid (see “The grid action bar” below) has an **Inactivate / Reactivate** button. Inactivating an encounter does not delete or hide it — it stays in the list, shown in grey, and you can select it again later and reactivate it.

Why “newest” means highest encounter number. Encounter lists are ordered by encounter number rather than by admission date. A small number of encounters brought in from older systems years ago carry admission dates that are plainly wrong — a year like 9201 or 4208 — and ordering by date put those at the very top, so the first thing you saw was a screen of bad data. Encounter numbers are issued in order, so they give the same “newest first” without that. The odd dates are still in those old records; they are simply no longer what greets you.

You can still sort by **Admitted** by selecting that heading, which is occasionally what you want — just expect the strange ones at the end.

1.6 The grid action bar

The patients list and both encounters grids (the standalone Encounters list and the patient’s own Encounters tab) each have a row of buttons in the bottom-left of the grid, next to the paging controls. Most buttons only work once you’ve selected a row — select a row first, then choose the action.

On the Patients list, the buttons are:

- **New** — start a new patient record. Works without selecting a row.
- **Edit** — open the selected patient’s record.
- **Encounters** — jump straight to the selected patient’s Encounters tab.
- **Claims** — open a read-only list of the patient’s claims.
- **Notes** — open the patient’s notes. Unlike Claims, Audit log, and Appointments, you can add a new note here (type it in the box and select **Add note**) — you cannot edit or delete an existing note.
- **Audit log** — a read-only history of changes made to this patient’s record, on the web or on the desktop.
- **Appointments** — a read-only list of the patient’s appointments.

Each of these opens in its own dialog on top of the patients list, so searching or paging the list underneath is undisturbed.

On an Encounters grid (the standalone list and the patient’s Encounters tab), the buttons are:

- **New** — start a new encounter. Works without selecting a row. **Only on the patient’s Encounters tab**, not on the standalone Encounters list. An encounter belongs to a patient, and the standalone list has no patient in hand — so it is created from the patient’s own record, where you can see whose visit you are creating. The desktop works the same way.
- **Edit** — open the selected encounter.
- **Orders** — record what the patient is in for. See **Orders**.

- **Inactivate / Reactivate** — flips the selected encounter’s status. You’ll be asked to confirm. An inactivated encounter is greyed out in the list, not removed, and can be reactivated the same way.
- **Capture signature** — see **Patient signatures from a signature pad**.
- **Print encounter, Print label and Preview** — see **Printing encounter forms and labels**.

Every one of these opens **in a window on top of the grid**, and the grid is untouched behind it — same filters, same page, same selected row when you close. That applies to patients and encounters alike, from either grid.

If you reach an encounter as a full page instead — a bookmark, or a link somebody sent you — the **Back to list** button returns you to wherever you came from rather than always to the same place.

Reset layout, in the same bar, restores the grid’s columns to their original widths and order. It leaves your search and filters alone.

1.6.1 Working an encounter

Two fields on the encounter form check what you type against the practice’s own lists rather than accepting anything.

Doctor. Type an id, or a surname, or a first name, and a short list of matches drops down — choose one with the mouse, or the arrow keys and Enter. The field beside it then shows **who that doctor is**, read-only. With nearly ten thousand doctors on file, an id on its own tells you very little, and this is how you confirm you picked the right one. Upper or lower case makes no difference to the search.

If you type an id that does not exist, the box turns red. Nothing stops you saving — it is a warning, not a barrier.

Admitting code. The same control, against the ICD-10-CM list. Type a code like **E11** to see the codes under it, or type words like **diabetes** to search the descriptions. The **Admitting description** beside it is filled in from whichever code you choose and cannot be typed into — a description that disagreed with its own code would be worse than none.

Leaving it blank is perfectly valid, and normal: this field has not been used before, so almost every existing encounter has it empty. It is checked only if you put something in it.

Only codes that can actually be billed are offered. The ICD-10 list contains group headings and parent codes as well — **E11** is a heading, **E11.00** is a real code — and offering the headings would invite choosing something that cannot be billed.

1.7 Orders — what the patient is in for

An encounter can carry one or more **orders**: the items from your practice’s catalogue that the patient is being seen for. You can open Orders from three places, and all three lead to the same screen:

- the **Orders** button on the patient’s Encounters tab,
- the **Orders** button on the standalone Encounters list,
- the **Manage orders** button on the encounter itself.

Orders are only available once an encounter has been saved. On a brand-new encounter, save it first and the Orders card appears.

The screen has two lists side by side. On the **left** is your catalogue — every item you can order. On the **right** is what this encounter has already been ordered. The arrows between them move items across:

- select an item on the left and choose → to add it,
- select an order on the right and choose ← to take it off again.

Nothing is saved until you choose **Save orders**. Until then you are only building up the list, and **Close** discards the lot. After saving you’ll see a short summary of what happened — for example “Saved: 2 added, 1 removed”.

To find something in a long catalogue, type part of the description or the CPT code in the search box above the left-hand list and select **Search**. Clearing the box and searching again brings the whole catalogue back.

An order that has been transcribed cannot be removed. Once a transcription has been written against an order, that order is part of the patient’s clinical record, and the system will not let you take it off the encounter. These orders are shown greyed out and in italics in the right-hand list, and choosing ← on one tells you why instead of removing it. If such an order really is wrong, the transcription has to be dealt with first — the order can then be removed normally. You can still add and remove other orders on the same encounter as usual.

If you ever see an order described as “**(catalogue entry missing)**”, the item it was ordered from has since been deleted from the catalogue. The order is still real and still on the encounter — it is shown this way deliberately, rather than hidden, so nothing disappears from the record without you knowing.

1.8 The Modality worklist

Modality worklist in the menu shows the imaging orders queued for the scanners — the same queue the imaging equipment itself reads when it asks “what am I scanning today”. It is a read-only view: nothing here is edited from the browser.

Each row is one scheduled procedure, with the patient’s name, the accession number, the modality (CR, CT, US and so on), the procedure, and when it is scheduled for.

- **Search** by patient name or accession number. Case does not matter — type **rivera** or **RIVERA**, whichever is quicker.
- **Status** is **Scheduled** by default. **Cancelled** shows what has been called off, and **All** shows both.
- **Modality** is a funnel in that column heading, and takes more than one at once — CT and MR together, say.
- **Dates** default to today, plus and minus a day, so a procedure scheduled late last night or early tomorrow is still in view. **Change them and your window is remembered**, like every other filter, until you close the browser. Leave them alone and they follow the calendar instead of freezing — a browser left open overnight opens on today, not on yesterday. **Clear** returns them to following the calendar.
- **Auto-refresh** keeps the list current without you doing anything. It is on by default; the tick box turns it off.

Auto-refresh deliberately holds back in two situations, so it cannot pull the screen out from under you: while you have a row selected, and while the browser tab is in the background. Select nothing and it resumes.

1.8.1 When the list is empty

An empty worklist is usually an ordinary answer, and the message says which kind it is rather than making you guess:

- **nothing scheduled in this window** — the normal quiet case;
- **nothing matches your filters** — you have narrowed it to nothing, so widen the dates or clear the search;
- **nothing in the table at all** — this one is worth reporting, because it can mean the process that fills the worklist has stopped rather than that there is genuinely no work.

The **Mp10 AutoTasks** guide covers what fills this table and how it is turned on, which is an administrator’s job rather than a desk one.

1.9 Capturing images

Both the patient ID card (Demographics tab) and insurance cards (Insurance tab) let you attach a photo.

- **On a desktop computer:** selecting “Capture / Upload” opens a file picker. Choose an image you’ve already scanned into a folder (the same scan-to-folder process your office already uses).

- **On a phone or tablet:** the same button can open your camera directly, so you can photograph the card on the spot.
- Accepted formats are JPEG and PNG, up to 4 MB. Large photos are automatically shrunk before they're uploaded, so a full-size phone photo is fine — you don't need to resize it yourself.
- Select **Replace** to swap out an existing image, or **Delete** to remove it.

1.10 Patient signatures from a signature pad

If a **Topaz signature pad** is attached to your computer, you can take the patient's signature straight from it.

Select the encounter on either Encounters list and choose **Capture signature**.

If your practice has set up a consent text, the window shows it first, one page at a time — **Continue** stays greyed out until the patient has reached the last page. If a choice was set up, they pick one of the options offered; it is one choice, not a tick-list. Then the signing area appears: the patient signs on the pad and their signature appears on screen as they write. **Clear** starts them over, **Save signature** stores it.

If the patient changes their choice after signing, the signature is cleared and they are asked to sign again — the signature has to belong to the choice they actually made. The window says so when it happens.

With no consent text set up, the window goes straight to the signing area.

The button only works for an encounter that has no signature yet. When it is greyed out, hover over it and it will say *"This encounter already has a signature"*. To replace one that is already there, open the encounter and use the **Signature** panel on the encounter form.

Two things it deliberately refuses:

- **Saving before anyone has signed.** It says so instead of storing a blank image — an encounter that looks signed but is not would be worse than one that is plainly unsigned.
- **Working on a Mac, tablet or phone.** The pad software is Windows-only, and the window says so rather than appearing to work. Use the **Signature** panel on the encounter form instead — that works everywhere.

If it reports that something is missing (`SigWebTablet.js`, or SigWeb not running), that is a setup task for whoever looks after the computers, not something you can fix at the desk. Nothing is saved when it happens, so the encounter simply stays unsigned. The full details are in the **Mp10 Signature Pads** guide.

1.11 Printing encounter forms and labels

Printing happens on **the printer attached to the computer you are sitting at**, using the same templates as the desktop application — so a label printed here and a label printed from Mp10 come out identical.

Select the encounter on either Encounters grid, then:

- **Print encounter** — the encounter form, to this computer's FORMS printer.
- **Print label** — the encounter label, to this computer's LABELS printer.
- **Preview** — exactly what would have been printed, opened as a PDF in a new browser tab, with nothing sent to a printer.

The printer is chosen automatically; nothing asks you which one.

Preview is worth knowing about. Use it to check a form looks right before committing a sheet, or when a label came out wrong and you want to see whether the problem is the data or the printer. It is the same template and the same data as printing, not an approximation. If nothing opens, your browser has blocked the new tab — allow pop-ups for this site; the app tells you when that has happened rather than appearing to do nothing.

The Print button on the Claims dialog is not finished. It reports that claim printing is still to be built. Print claims from the desktop application in the meantime.

Designing and editing the report templates stays in the desktop application, deliberately: a template is shared by everyone in the practice, and editing one is a deskbound job.

1.11.1 When printing does not work

Nothing is ever half-printed: when it fails, nothing was sent, and the message on screen names the cause. Most of them come down to the printer not being found by name, or the print helper not running on that computer — both setup tasks rather than something to fix at the desk.

If a print job stops responding, Mp10 Web refuses further printing from that computer until the helper is restarted, which is better than queueing work that is never going to come out. Somebody with administrator access can do that from **Admin** → **Print helper** in the menu, without leaving the browser; that page also lists which printers the helper can actually see, which is what solves most problems.

The full account — what each message means, how the helper is installed and configured, and how to work through a fault — is in the Mp10 Web — Printing guide. It is not repeated here, so that there is only ever one version of it to keep up to date.

1.12 Who made a change

Every change made through Mp10 Web is recorded under the web user's own operator code, the same way desktop changes are recorded under a desktop operator code. This means a colleague using the desktop application can see that a change came from you, from the web app, just like they would for any other change.